

ST. AUGUSTINE ORTHOPEDIC CENTER

Department of Orthopedic Surgery

DISCHARGE SUMMARY

Document No: DS-2024-01823	Admission Date: March 10, 2024	Discharge Date: March 16, 2024
Attending Physician: Dr. James Okafor, MD, FAAOS		

Patient Name: Robert Eugene Hastings	Date of Birth: February 17, 1943 (Age: 81)
MRN: SAOC-443-8821	Sex: Male
Insurance: Medicare Advantage — BlueCross	Discharge Disposition: Skilled Nursing Facility — Cedar Brook Manor

PRIMARY DIAGNOSIS

Right total hip arthroplasty (THA), primary

Secondary: Type 2 diabetes mellitus (A1c 8.2%), Peripheral arterial disease, Chronic low back pain

REASON FOR ADMISSION

Patient admitted electively on 3/10 for right total hip arthroplasty secondary to severe osteoarthritis with functional limitation. Patient had failed conservative management including physical therapy and corticosteroid injections x2. Preoperative workup including cardiac clearance completed by PCP Dr. Rivera on 3/5/2024.

HOSPITAL COURSE

Patient underwent uncomplicated right THA under spinal anesthesia on 3/11. Estimated blood loss 220mL. Intraoperative and postoperative course uneventful. Patient mobilized with physical therapy POD1, ambulating with walker POD2. Blood glucose management required supplemental insulin during admission. Wound site clean, dry, and intact at discharge. DVT prophylaxis initiated with enoxaparin.

DISCHARGE MEDICATIONS

Medication	Dose	Route	Frequency	Notes
Enoxaparin (NEW)	40mg	SubQ	Once daily x28 days	DVT prophylaxis — CRITICAL do not miss
Oxycodone/APAP (NEW)	5/325mg	PO	PRN q4-6h pain	Max 4g APAP/day — taper after 5 days
Acetaminophen	650mg	PO	q6h scheduled	INCLUDED in Oxycodone/APAP — monitor total dose
Metformin	1000mg	PO	Twice daily	Resume now — was held perioperatively
Glipizide	5mg	PO	Once daily with breakfast	Resume — was held perioperatively
Gabapentin (NEW)	300mg	PO	Three times daily	For post-op pain — taper after 2 weeks
Aspirin	81mg	PO	Once daily	Continue — do NOT double with enoxaparin
Atorvastatin	20mg	PO	Nightly	Continue unchanged

WOUND CARE INSTRUCTIONS

Surgical staples in place — scheduled removal at 14 days post-op (3/25/2024).

Keep wound dry for 48 hours. May shower after 48 hours — pat dry immediately.

Inspect wound daily for signs of infection: increasing redness, warmth, swelling, purulent drainage, or fever >101.5F.

No submersion in water (bath, pool, hot tub) until wound fully closed.

[!] Wound dressing change frequency NOT specified in discharge orders — facility to confirm with surgeon.

[!] Staple removal appointment: NOT scheduled — facility must arrange with Dr. Okafor's office at 617-555-0847.

MOBILITY & ACTIVITY RESTRICTIONS

Weight bearing as tolerated (WBAT) right lower extremity with walker.

Hip precautions: No hip flexion >90 degrees, no internal rotation, no adduction past midline.

Physical therapy: 5x per week at SNF — order placed.

Occupational therapy: 3x per week — order placed.

ALLERGIES

Codeine — nausea/vomiting

Latex — contact dermatitis

DIABETES MONITORING

Check fingerstick blood glucose before meals and at bedtime.

Notify provider if glucose >300 or <70 mg/dL.

Sliding scale insulin order attached (see addendum).

[!] Sliding scale addendum referenced but NOT included in discharge packet.